# HIV Screening Clinical Quality Measure Report

Monthly Clinical Quality Metrics Report | Measurement Month: June 2026 | Created by: Skye Johnson - WEA Public Health Intern & Innocent Menyo - WEA Data Analyst
Report date: June 30, 2026 | Data source: Athena + LabCorp (June 1, 2025 – June 30, 2026)

## Measure Definition

**Clinical Quality Metric (CQM)**: For WEA patients who have at least one medical visit in the measurement month (denominator), how many have an HIV test result in the last 12 months (numerator; # and % of denominator)?

The denominator captures patients with at least one non-cancelled appointment in June 2026, restricted to ages 15-65 years at the time of testing per Uniform Data System (HRSA UDS) CMS349v5. The numerator identifies, among those patients, those who have a documented HIV screening test result anywhere in the 12-month lookback window (June 1, 2025 – June 30, 2026) per the CPRC guide. The measure applies across all WEA regions (Coachella Valley, San Bernardino, Los Angeles).

## Why Is This Metric Important for WEA?

The HIV Clinical Quality Measure is important to Wellness Equity Alliance (WEA) because it aligns directly with the organization's mission, vision, and service delivery model. WEA is committed to providing equitable, high-quality healthcare to marginalized populations, particularly individuals impacted by homelessness and those affected by behavioral health conditions. These populations are disproportionately impacted by HIV and often face barriers to accessing preventive healthcare services.

The mission of providing equitable healthcare is backed by WEA services that include behavioral healthcare and crisis walk in center, mobile healthcare delivery to rural communities, and street medicine treatments to unhoused individuals. A particular service and initiative started at WEA is HIV street medicine, which offers HIV testing, PrEP delivery, hygiene supplies, and harm reduction services to combat the U.S. HIV epidemic.

Monitoring the HIV Clinical Quality Measure monthly aids in the organization-wide HIV street medicine initiative to evaluate whether eligible patients are receiving recommended HIV screenings and whether gaps lie in the facilitation of preventive care. Ultimately, these quality improvement efforts help ensure equitable access to HIV screenings for underserved communities and WEA's commitment to eradicating the HIV epidemic.

### June 2026 Results

| DENOMINATOR | TESTS ORDERED | NUMERATOR (COMPLETED) | RATE |
|---|---|---|---|
| **123** | **2** | **2** | **1.6%** |

| DENOMINATOR | TESTS ORDERED | NUMERATOR (COMPLETED) | RATE |
|---|---|---|---|
| Visits in June 2026 (ages 15–65; UDS criteria) | HIV tests ordered in the lookback period | 2 / 123 with a completed HIV Ab/p24 Ag Screen | 2 /123 |

**Note on June 2026 Results:** To create and interpret the results of the clinical quality measure for June 2026, R studio (version 4.5.2) was used.

**Note on incomplete test**: Of the 2 HIV tests ordered, there were no incomplete tests due to an insufficient specimen. The 2 patients with a completed HIV Ab/p24 Ag Screen result constitute the numerator.

**Test name used**: All 2 completed screening tests are recorded under the test name HIV Ab/p24 Ag with Reflex, with the analyte name HIV Ab/p24 Ag Screen in the LabCorp data. This is an antigen/antibody combination test and qualifies as an HIV screening test for the purposes of this measure.

## Benchmark Comparison

| Benchmark / Source | Rate | Population | Notes | WEA Status |
|---|---|---|---|---|
| **WEA - June 2026** | **1.6%** | WEA patients with June 2026 visit, ages 15–65 | 2 of 123 patients have a completed HIV screen. | **Critical gap** |
| HRSA UDS National Avg. (2023) | ~62% | FQHC patients ages 15–65 | CMS349v5; most recent published national average from HRSA BPHC. | **−60 pts** |
| HRSA UDS Top Quartile (NQL) | ~80% + | FQHC patients ages 15–65 | National Quality Leader badge threshold; adjusted quartile ranking. | **−78 pts** |

| | | | | |
|---|---|---|---|---|
| HEDIS (MY 2026) | N/A | Commercial / Medicaid enrollees | No standalone HIV screening rate measure. HIV measures in HEDIS are monitoring measures for known-positive patients only. | Not applicable |

***pts - Percentage Points***

**Note:** HEDIS does not publish a direct comparator for annual HIV screening rate. The primary external benchmark for this WEA measure is the HRSA UDS CMS349v5 national average. Age restriction for the denominator follows UDS criteria: patients 15–65 years of age.

## Understanding Benchmark Comparisons

WEA's June 2026 HIV screening rate was 1.6% (2 of 123 eligible patients), substantially below the HRSA UDS national average of approximately 62% and the estimated top-quartile performance of approximately 80% or greater. These comparisons indicate a significant quality gap and highlight the need for targeted interventions to increase routine HIV screening among eligible patients. Because HEDIS does not include a standalone measure for routine HIV screening in the general population, the HRSA UDS CMS349v5 measure serves as the most appropriate benchmark for evaluating WEA's performance. A limitation of note is that this benchmark comparison to WEA's HIV screening rate is based on a single month, while the HRSA UDS benchmarks are based on annual organization-level rates.

## Improvement Tactics

### Tactic 1 - Adopt opt-out testing language [In Progress]

Train all clinical staff to present HIV testing as a routine, expected part of care. Script development and staff training materials to be coordinated through CPRC. Opt-out framing aligns with CDC and USPSTF guidelines and is associated with significantly higher screening uptake in community health settings.

### Tactic 2 - Tie-in with initial medical visits [In Progress]

Embed HIV screening into the workflow of all initial provider visits across sites. For patients who have already completed initial visits without screening, schedule nurse visits to close testing gaps.

### Tactic 3 - Patient incentives [Under Evaluation]

TBD by CPRC

## Expanded Improvement Tactics

### Tactic 1 - HIV Screening Initiative [Proposed]

Create specified office/street medicine coordination days which focus on promoting and facilitating HIV

screenings. This can be done annually, such as on HIV awareness day or other dates, as a conjoined effort across all WEA offices to increase the uptake of HIV screenings. Patient incentives, which have been already proposed as an improvement tactic, can also be paired with this initiative.

### Tactic 2 - Patient Education (Tie-in with opt-out testing language) [Proposed]

WEA largely provides care to individuals who are at risk for HIV and other infectious diseases. These individuals include those who are unhoused and facing substance abuse. With this high-risk population, it is essential that WEA staff address any gaps that lie between the need for HIV screenings and patient perception of such screenings. To address patient education regarding HIV screenings, Opt-out testing script and staff training materials should include plain-language, culturally responsive communication strategies that reflect the health literacy needs of the populations served by WEA offices. Providers and staff should be prepared to address common misconceptions and concerns as well as internal bias regarding HIV screening. This initiative should be provided during all patient encounters, regardless of the initial reason for seeking treatment.

### Tactic 3 - Embedded Clinical Decision Support [Proposed]

Configure electronic medical record systems at each WEA location to identify patients who are due for HIV screening based on current CDC recommendations and display reminders during eligible encounters. Incorporate HIV screening status into pre-visit planning, so care teams can proactively address overdue screenings before the provider enters the exam room. Documentation fields should also allow providers to indicate whether testing was completed, declined, or deferred, improving follow-up for HIV screenings and quality reporting.

### Tactic 4 - HIV Screening Implementation Monitoring & Performance Evaluation [Proposed]

Following the implementation of in-progress tactics, regularly monitor HIV screening rates by clinic location and patient population to identify variation in performance. Review missed screening opportunities, patient deferrals or declinations, and workflow barriers. Sharing performance data with WEA leadership and care teams can support continuous improvement and help identify needed modifications. Quarterly and Yearly Clinical Quality Measure Reports should also be developed to understand periods of gaps or success in HIV Screening performance. Yearly HIV screening rate monitoring will also depict a more precise comparison of WEA's screening rate to HRSA UDS benchmark standards.

## Methodology & Data Notes

**Reporting period**: The measurement month is June 2026. The HIV screening lookback window covers June 1, 2025 to June 30, 2026, 12 months preceding the reporting month. All LabCorp lab results used in the numerator assessment are drawn exclusively from this date range.

**Denominato**r: WEA patients with at least one non-cancelled appointment in June 2026, restricted to ages 15–65 per UDS CMS349v5. Final denominator = 123. Deduplicated on patient ID.

**Numerator**: LabCorp lab data (June 1, 2025 – June 30, 2026) merged with appointment data by patient ID (full join). HIV screening tests identified from the analyte_name field. The qualifying test name is HIV Ab/p24 Ag with Reflex (analyte: HIV Ab/p24 Ag Screen) — an antigen/antibody combination test. Final numerator = 2 patients with a completed HIV screening result.

**Rate calculation:** 2/123 = 1.6%

**Age restriction:** Per UDS CMS349v5 specifications, the denominator is restricted to patients ages 15–65. Patients aged 14 and under, or 66 and older, are excluded from both numerator and denominator.

**Excluded tests:** Viral load tests (HIV-1 RNA, HIV-2 RNA, log10 HIV-1 RNA) and disease monitoring tests (CD4 count, HIV GenoSure) were excluded as they do not constitute HIV status screening and are not applicable to this measure.